Historical US Product Classification Assessment

Original assessment: October 10, 2026 (America/Chicago).

The original assessment pages follow unchanged. Their older app-status statements, proposed commercial alternatives, and source-verification limits reflect the original assessment date. They do not describe the current software implementation or selected MVP.

The selected direction is an integrated physical-device, companion-app, and model research MVP leading toward a pocket consumer product. The existing-products/refill service discussed in the original assessment remains an alternative commercial route; it is not the selected MVP.

For current evidence and product scope, see the implementation status and selected integrated research MVP. These documents do not establish a validated assay, qualified human sensing configuration, clinical model, or consumer release.

Editable historical assessment: CLASSIFICATION_OPTIONS.md. This cover is an archival annotation; it does not alter the substantive original assessment.

Product classification with less regulatory work

One-The-Go Detection · United States · Consumer self-assessment
Prepared October 10, 2026 (America/Chicago)

Your stated goal: people use the product themselves to judge whether they are impaired. Consumer control of the final decision does not automatically make an impairment assessment exempt or “wellness.”

What the repository establishes

The repository describes a proposed pocket device and mobile app combining saliva THC measurement, a reaction task, pupil capture and cardiac signals, with a separate future breath-alcohol channel. It is currently a specification and synthetic integration starter. It demonstrates no validated THC assay, production hardware, working mobile app or impairment model.

This assessment reviewed the README, PRD, decision log, existing FDA assessment and faster-market strategy, and the implemented synthetic tools. The United States and consumer self-assessment are confirmed by you.

Recommended route—if a narrower first release is acceptable

For the least new device regulatory work, start as a **domestic distributor of unchanged, legally marketed finished tests**, alongside an app limited to ordering, refills, general education and faithful, source-attributed result history. Confirm the exact products’ consumer authorization, labeling, supplier relationship and supported workflow. Preserve manufacturer packaging and instructions.

This service would not tell a consumer whether they are impaired. If that output is mandatory for the first release, the full inference function must be assessed on its actual intended use and evidence. No verified exemption for that exact function has been established here.

If a proprietary pocket instrument is essential at first sale, evaluate a narrower exposure-screening product with a qualified OEM. Keep individualized impairment prediction as a separate research program. A cleared cartridge does not automatically authorize a new collector, reader, app control or integrated configuration.

These are scope recommendations, not an FDA classification determination. Live primary-source verification was blocked; see page 3 for the evidence limits.

Compare the first-release options

The regulatory role, software function and finished-device claim must each be assessed.

First offering	Provisional regulatory direction	Work saved and limits
Administrative / history app only	Potential non-device functions, assessed individually.	Avoids developing a test or inference engine. Privacy and truthful marketing still apply.
Unchanged finished tests from a US supplier + limited app	Domestic distribution plus potentially non-device software.	May avoid a new manufacturer submission for unchanged activities. Importing, relabeling, repackaging and kit assembly require separate review.
Own oral-fluid THC screen	Repository identifies Class II cannabinoid testing under 21 CFR 862.3870; assess the OTC submission route.	Narrower than impairment, but a chemical screen is not automatically unregulated.
Separate breath-alcohol measurement	Repository identifies Class I under 21 CFR 862.3050, conditionally 510(k)-exempt.	Exemption limits and other obligations remain. This does not exempt THC functions.
THC + physiological / task impairment score	Exact classification and predicate remain unresolved.	Highest evidence uncertainty. No exemption for the proposed consumer impairment function has been established.

Concrete scope for the lowest-burden service

Proposed description: **“Manage results and refills for supported THC screening tests and breath-alcohol products.”** Match every claim to the actual supplier labeling. Begin with manual, attributed result entry.

Exclude automatic strip reading, new device control, pupil/cardiac interpretation, personalized intoxication scores and clearance countdowns from that release. Calling unchanged impairment functionality wellness, entertainment, beta or research does not itself change its intended use.

An employment/insurance-only exemption discussed in the repository would require a genuinely different intended use and distribution; its current conditions still need verification. It cannot be borrowed for the consumer self-test. Supplier authorization does not substantiate driving fitness or a combined impairment conclusion.

Decisions and source verification

Resolve before selecting the route

1. Freeze the intended use. Define the exact result, intended adults, use setting and decisions it supports. Specify whether “am I impaired?” is required at first launch and whether use includes driving or other safety decisions.

2. Define the commercial configuration. Decide whether one custom instrument is mandatory; identify the first state, exact supplier and product; and establish whether your company distributes domestically, imports, relabels or specifies the finished device.

3. Prepare a function/activity matrix. Separate the assay, reader, app history, device control and impairment inference. Compare each with its actual regulatory basis before deciding whether an FDA information request is needed.

Primary sources to verify

1. **FDA: who must register, list and pay the fee**
Relevant to domestic distribution and changes in business role.

2. **FDA: examples of software functions that are not medical devices**
Relevant to faithful storage/display and limited administrative functions.

3. **21 CFR 862.3870: cannabinoid test system**
Classification and any claim-specific exemption conditions.

4. **21 CFR 862.3050: breath-alcohol test system**
The separate breath-alcohol category.

5. **21 CFR 862.9: limitations of exemptions**
Limits on relying on a premarket exemption.

6. **FDA: K252523 decision summary**
OTC comparator cited in the repository; not independently confirmed here.

Evidence limits

FDA and eCFR retrieval attempts returned proxy network-policy denials. Specific 2026 regulatory developments and clearance claims in the existing repository reports were not independently verified in this session. All classification statements above retain that limitation. This assessment does not establish a final class, predicate, submission route or commercial authorization.

Repository references: README.md; specs/PRD.md; specs/DECISIONS.md; specs/research/FDA_REGULATORY_ASSESSMENT.md; specs/research/FAST_MARKET_STRATEGY.md. The source text for this assessment is stored outside the checkout in onboarding-research/CLASSIFICATION_OPTIONS.md.